BUSINESS MODEL CANVAS

NutriSense & N.E.X.U.S.

Designed for: early stunting prevention and safe pregnancies in rural NTT
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8 Key Partners

- **Dinas Kesehatan**: NTT province and districts
- **Puskesmas** and **posyandu**
- **Kemenkes**: SATUSEHAT platform
- **Village governments**: village funds, locker sites
- **BINUS International**: research and evaluation
- **Nutrition NGOs** and development partners: PMT, Taburia, RUTF
- **SIMPUS and hospital vendors**: FHIR link
- **Locker and logistics** providers
- **Telcos**: data, SMS
- **Cloud and AI** providers

7 Key Activities

- Build and maintain the app and API, tested
- **Clinical validation** and model retraining
- **Training** Kaders, bidan and Dinkes
- FHIR and SATUSEHAT **integration**
- **Logistics**: stock, routes, pickups
- Data protection (UU PDP), impact reports

6 Key Resources

- **Offline-first app** and back end
- WHO and Buku KIA rules, **explainable AI**, Nuri
- **HL7 FHIR R4** integration
- **Kader and Puskesmas network**
- Smart **lockers** and supply hubs
- Consent, audit, encryption: **trust**

2 Value Propositions

MOTHERS

- "Is my child growing well?" **in plain words**, and the one thing to do next
- **Danger sign → help now**, even with no signal
- Cheap, local meals from what is at home (NutriScan)
- A pregnancy companion, from HPHT to nifas

KADERS AND CLINICIANS

- Guided, fast measuring; **who to visit first**
- Danger reports stay pinned **until someone acts**
- Puskesmas results reach the mother's phone

DINKES AND FUNDERS

- **Early warning** before stunting shows; a risk map
- Supplies reach the right child, with proof of pickup

Offline first · Safety first · Explainable AI · FHIR integrated · One system

4 Customer Relationships

- **Kader-assisted** onboarding at the posyandu
- **Nuri**, an offline helper in simple Indonesian
- Transparent AI with **clinician review**
- Consent and privacy controls
- **Co-design** through pilot feedback
- Account management for the Dinkes

3 Channels

- **Posyandu** and Kader meetings
- **Puskesmas** and Dinkes introduction
- Android app or APK; web app
- **FHIR API** for SIMPUS vendors
- Pilot MoU, demo video, events
- In-app guide and helpdesk

1 Customer Segments

USERS (ALWAYS FREE)

- **Mothers and caregivers** of children under 5
- **Pregnant mothers**, through nifas
- **Posyandu Kaders**
- Bidan and doctors

PAYERS

- **Dinas Kesehatan** and Puskesmas
- **Village governments**
- Nutrition NGOs, CSR

🎯 **Beachhead: one Puskesmas area in rural NTT**

9 Cost Structure

Value-driven, with low marginal cost: the core runs offline and AI calls are optional

- **Product and engineering team** (fixed, largest)
- Cloud hosting, database, backups (small)
- AI API, optional (offline fallback)
- SMS and push notifications (planned)
- **Smart lockers**: capital, then upkeep
- Transport and restocking per package
- **Training** and field support per area
- Clinical validation, legal, UU PDP

5 Revenue Streams

H = hypothesis to test in the pilot

- **Platform licence** per Puskesmas area, yearly H
- Implementation and training H
- **Logistics fee** per package delivered H
- **Locker sponsorship**: villages, CSR H
- FHIR integration setup H
- **Grants** for the pilot and validation

Free forever for families and Kaders · no sale of personal data, ever

★ Social impact

D/S ↑	≤ 2 h	≥ 5 of 8	K6 ↑	Pickup rate ↑
children measured monthly	danger sign → contact	food groups a day	antenatal visits completed	supplies reach the child

Details, evidence and hypotheses: docs/BUSINESS_MODEL_CANVAS.md · Requirement IDs: docs/PRD.md

Based on the Business Model Canvas by Strategyzer (CC BY-SA 3.0)